PHARMATARGET | DOAC FIELD STRATEGY

Target the gap, not the volume.

Same 60 reps, same cost: re-target the calls, redraw the map, and measure what a call is worth before changing headcount.

01 **Re-target**
1.19x share gains

02 **Redraw**
106x -> 3.6x imbalance

03 **Pilot**
90% power, 2 quarters

Steering committee | decision pack | 10 slides

Re-target and redraw now; hold headcount and let a two-quarter pilot decide it

DO NOW

Re-target the 60-rep list

50/50 opportunity + volume blend. In held-out 2024 it won 1.19x the share gains of today's list and kept 91% of its absolute growth; it clears the 90% floor in 86% of re-draws.

DO NOW

Redraw the territories

Workload imbalance 106x -> 3.6x, every territory contiguous, travel 70% shorter. Pure arithmetic: no behavioural assumption.

TEST FIRST

Hold 60 reps, run the pilot

No measurable call response in public data. A 20% hold-out scored on share points detects the break-even response with 90% power in six months, and has the lowest worst-case regret of any headcount option.

$4.2M / yr

re-targeting, zero incremental cost

$4.5M

pilot vs deciding blind, 2 yrs

>= 1.48x

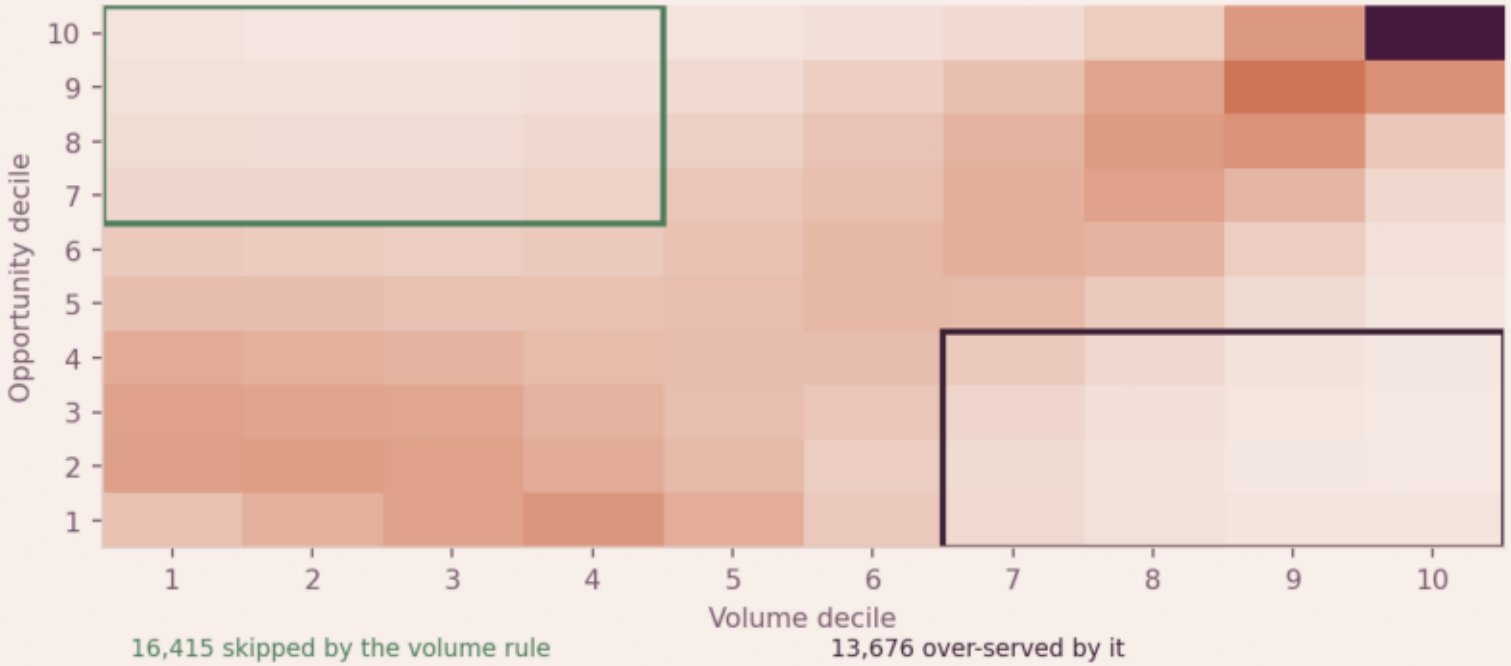
belief needed to justify hiring now

$0

new headcount until the pilot reads out

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

Volume ranking and opportunity disagree for 52% of prescribers: the default calls on doctors with nothing left to win



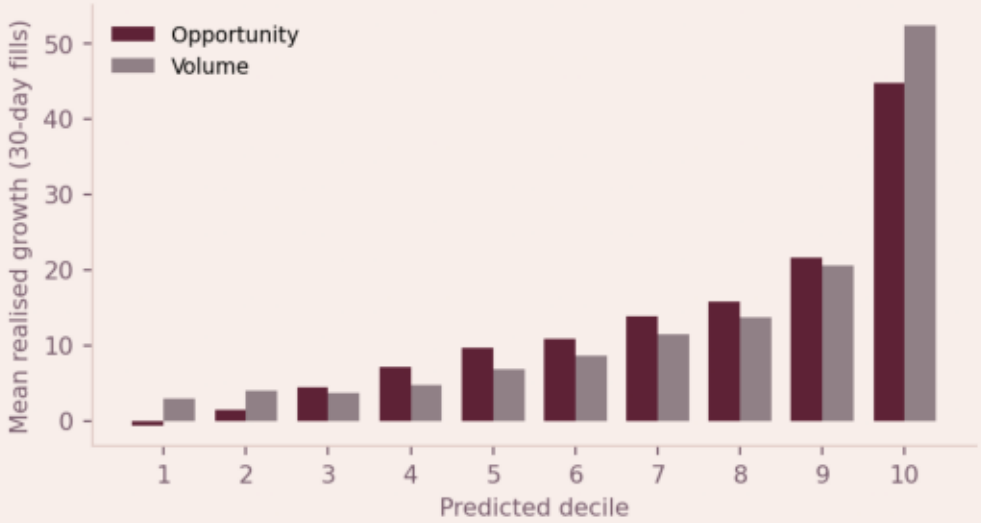
WHAT IT SHOWS

1. 16,415 low-volume, high-opportunity writers are skipped by the volume rule.
2. 13,676 high-volume writers with little headroom are over-served.
3. Opportunity = what a strong comparable prescriber reaches, minus what this one gives the brand today.

SO WHAT The list, not the headcount, is the first lever: it is free to change.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

In a year the model never saw, flagged prescribers grew 1.40x as fast as matched peers



WHAT IT SHOWS

1. Fit on 2023, frozen, scored on 2024; 39,727 propensity-matched pairs, worst covariate imbalance 0.053.

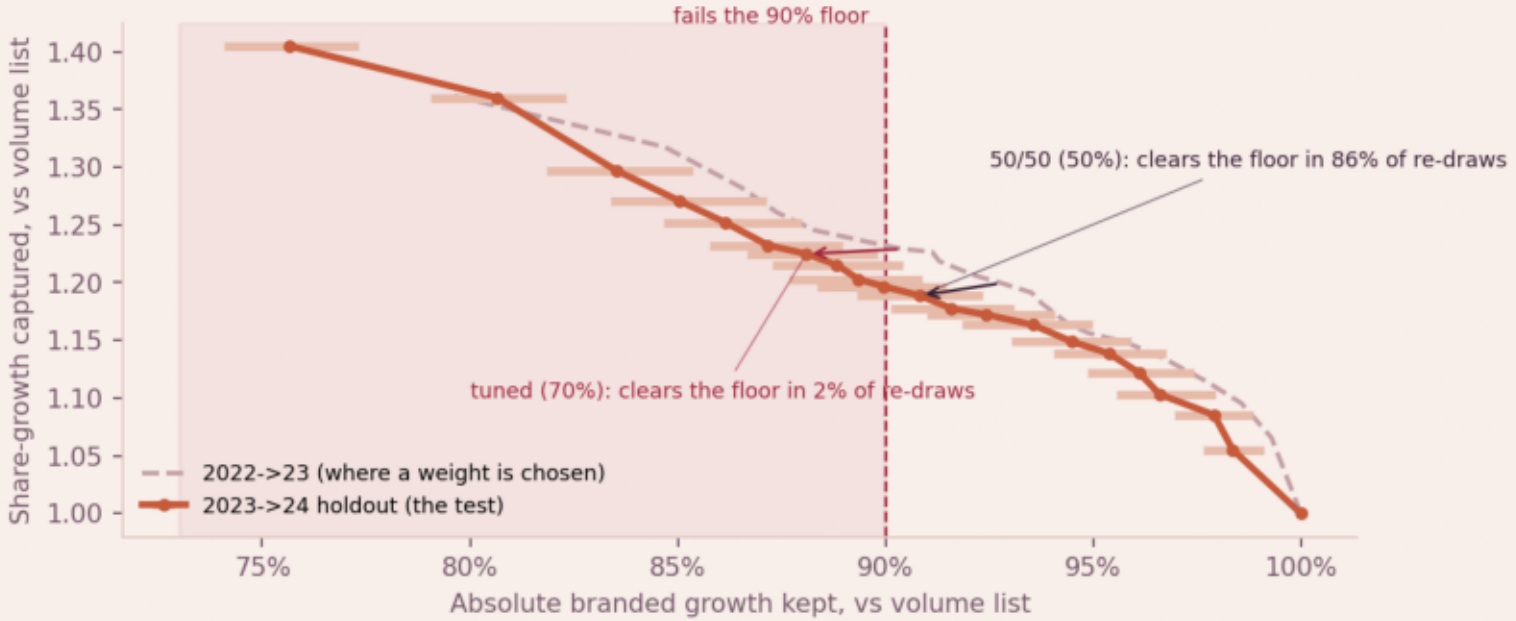
2. +4.9 branded fills per prescriber [3.8, 6.0].

3. It shows where growth happened, not what a call caused: that is the pilot's job.

SO WHAT The ranking finds growing prescribers out of time; it is not fitted to the exam.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

A 50/50 blend wins share without giving up the base; a weight tuned on last year would have failed this year



WHAT IT SHOWS

1. Rule fixed before the comparison: most share gain while keeping >= 90% of volume's growth. The blend: 1.19x share, 91% kept.

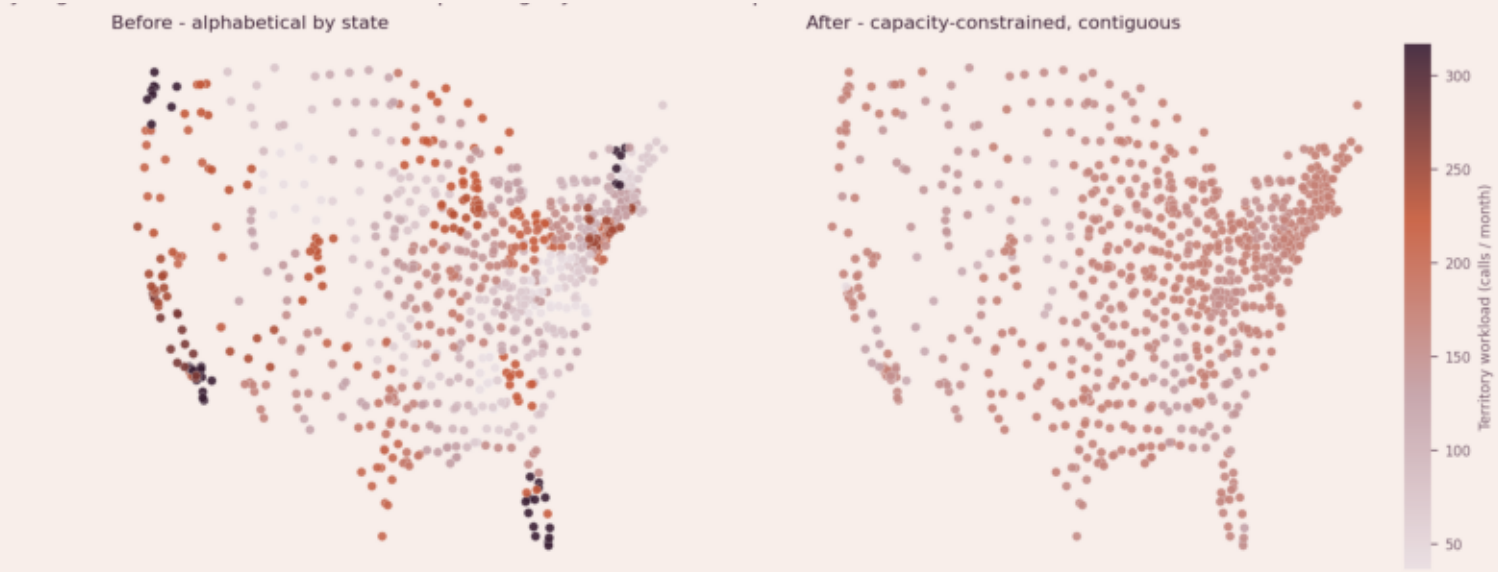
2. Tuned on 2022->23, the best weight (70%) clears the floor in 2% of 2024 re-draws; 50/50 in 86%.

3. For 95% confidence, use 45% at a small cost in share.

SO WHAT Adopt 50/50 and re-read the floor at every annual data refresh.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

Redrawing cuts workload imbalance from 106x to 3.6x with every territory contiguous



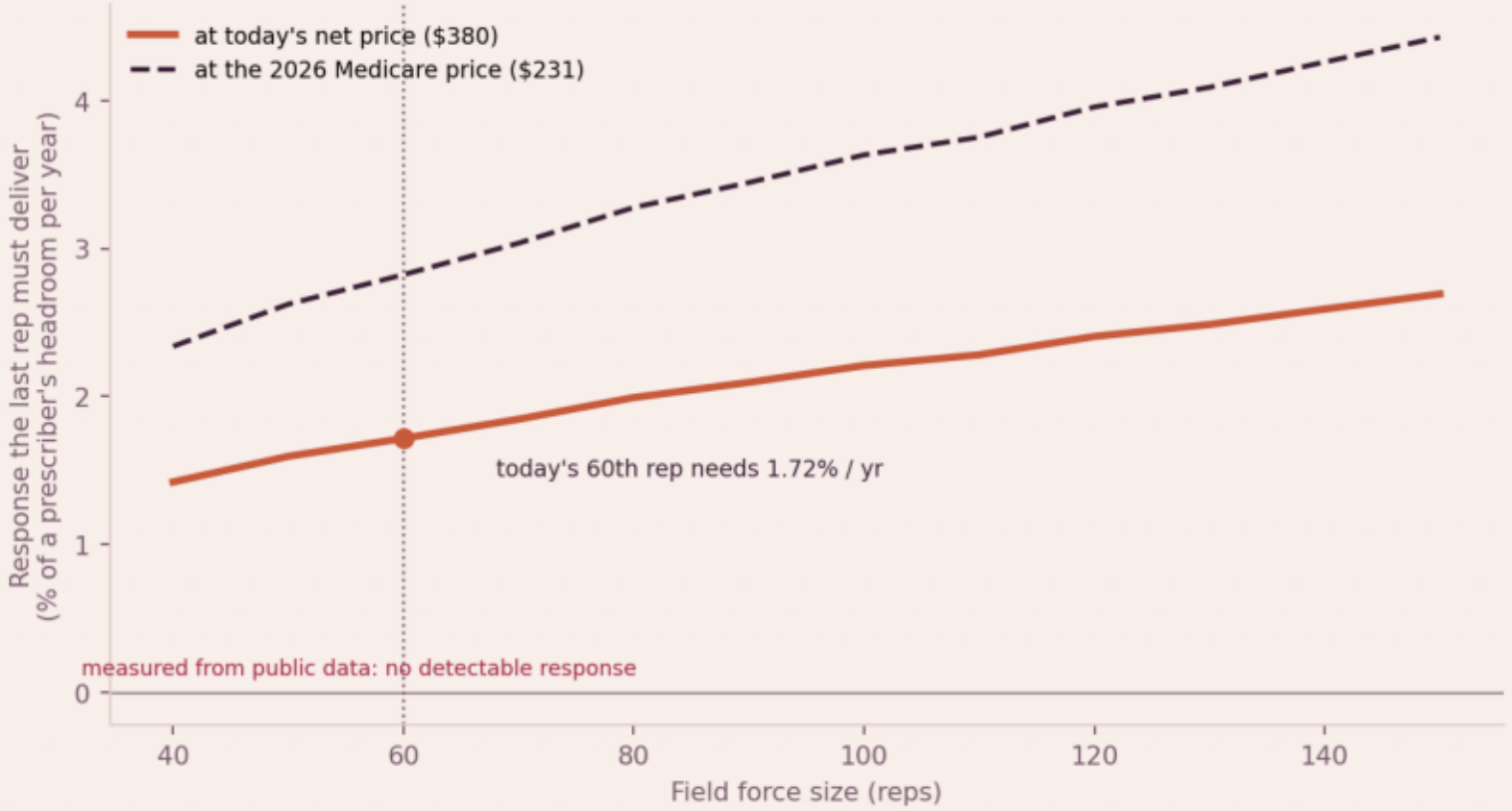
WHAT IT SHOWS

1. Contiguity 20% -> 100%; travel 70% shorter.
2. Plain k-means is tighter but leaves 523x imbalance: compact is not the same as workable.
3. Same 60 reps: no hiring, no new cost.

SO WHAT Redraw alongside the list change so reps meet the new targets inside a fair territory.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

Today's 60th rep pays only if a year of calls captures 1.72% of a prescriber's headroom, a number public data cannot measure



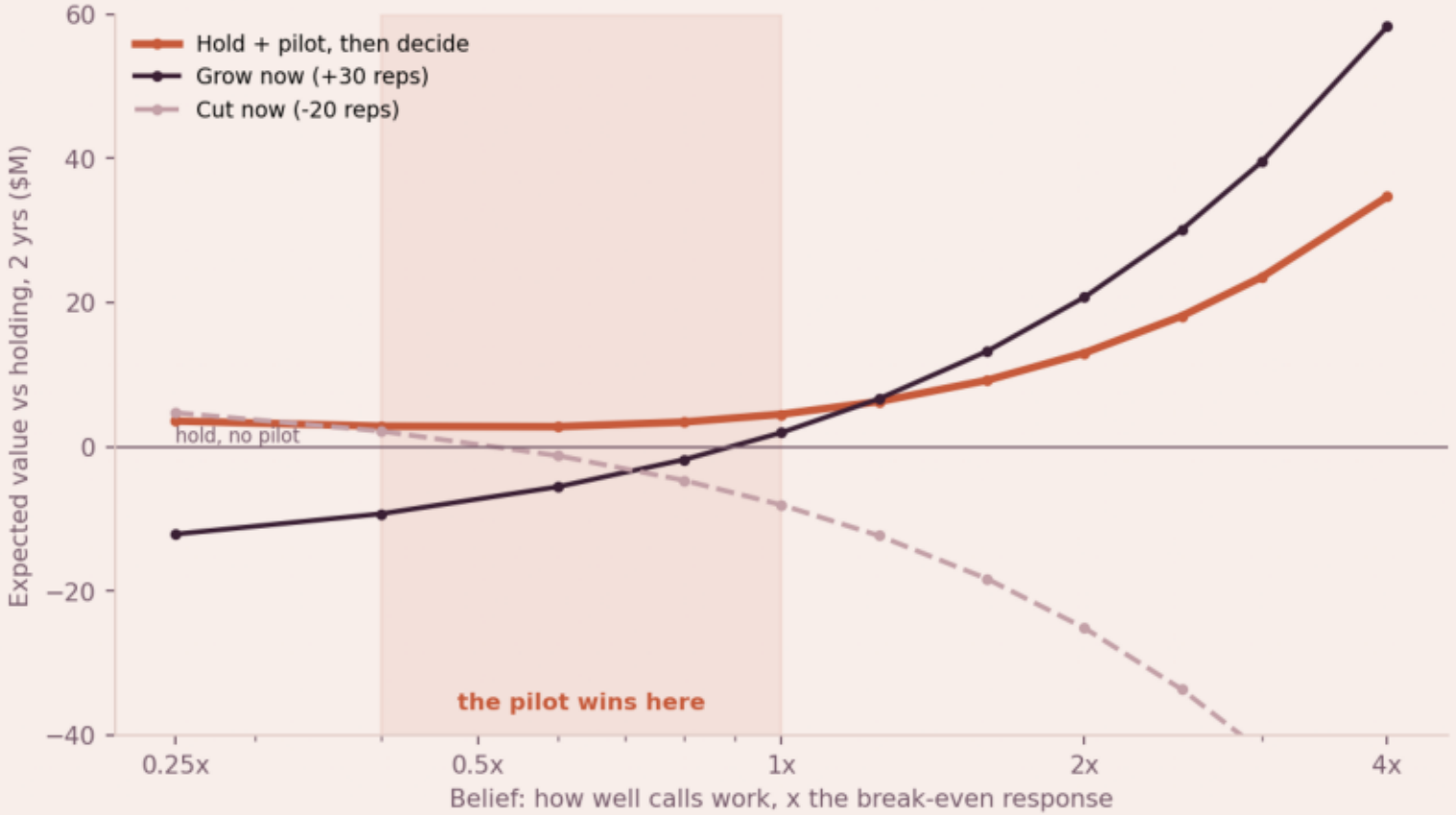
WHAT IT SHOWS

1. Brand-named visit events show no dose-response once prescribers are compared within volume bands.
2. So headcount is not priced on an invented curve; the required response is.
3. The 2026 Medicare negotiated price ($231 / 30 days) raises that bar ~1.6x.

SO WHAT Do not hire or cut on a number nobody has measured. Measure it.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

For any belief from 0.4x to 1.0x break-even, piloting beats every decision made blind



WHAT IT SHOWS

1. Re-targeting is worth $4.2M a year at the break-even response, at zero incremental cost.

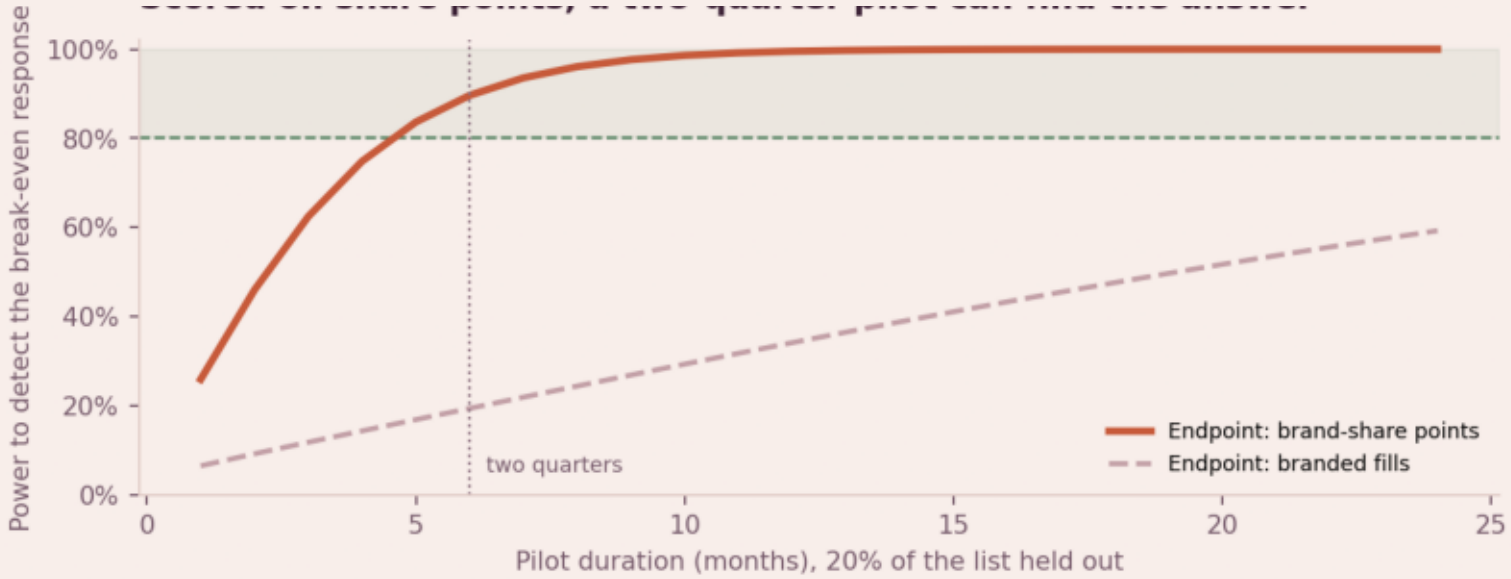
2. Hold + pilot is worth $4.5M over two years vs holding blind, net of $1.7M forgone during the hold-out.

3. Lowest worst-case regret of any option ($13.4M); hiring pays only if calls beat break-even by 1.48x.

SO WHAT The pilot is both the expected-value choice and the robust one: approve it.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

Scored on brand-share points, a two-quarter hold-out can find the answer; scored on fills it cannot



WHAT IT SHOWS

1. Hold out 20% of the list (no calls) for two quarters: 90% power on share points vs 19% on fills.

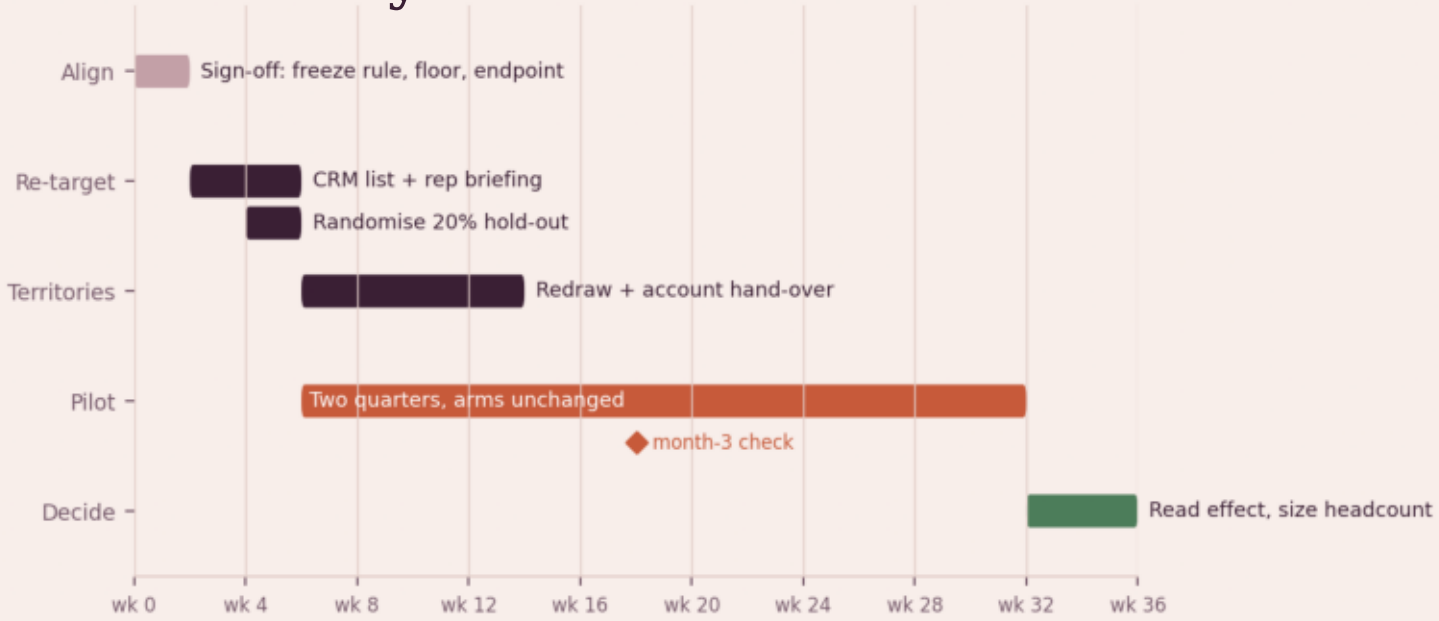
2. Fills are dominated by practice size; share points are volume-neutral.

3. Read at month six: < 0.5x break-even shrink, > 2x reopen hiring.

SO WHAT Fix the endpoint before launch: the choice of metric decides whether the pilot works.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.

Thirty-six weeks to a measured headcount decision; three approvals needed today



KILL CRITERIA

IF Pilot effect < 0.5x break-even at month 6 -> redeploy or shrink through attrition

IF Pilot effect > 2x break-even -> reopen headcount with a measured response

IF Blend keeps < 90% of volume's growth at a refresh -> move to 45% weight

IF Call compliance < 80% on the new list -> fix adoption before reading any result

DECISIONS NEEDED TODAY

1 Approve the blended call list

Same reps, same budget; CRM change in four weeks.

2 Approve the territory redraw

Phased hand-over; top accounts stay with their rep.

3 Freeze headcount at 60 for two quarters

And fund the 20% hold-out pilot; read-out at month six decides the size.

Source: CMS Medicare Part D & Open Payments, 2022-2024; pipeline output (data/manifest.json). Client and brand illustrative.